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Received: 13/Oct/2026 13:13:43
Verified: 15/Oct/2026 00:36:37

Name: ALEXANDER WILLIAM PETER THORP JIGGINS	Collection Date : 13/Oct/2026 09:10:00	Doctor: REFERRAL (VALEO WELLBEING DUBAI)
Sample Id: D26286061	Passport No:	Emirates ID: 784199020639185
Patient Id: MR01330101	Nationality: BRITISH	Center: VALEO WELLBEING DUBAI
Age/Sex: 36 Years/Male	Telephone: 971000000000	Clinic Barcode : VALEO-219231
DOB: 10/Jan/1990	File No: 2303	E-mail:
Fasting: Yes		

Test Name	Result	Flag	Units	Ref. Val.
✓Hemoglobin A1c	5.40		%	Less than 5.7 Normal (5.7 - 6.4) Prediabetes More than 6.5 Diabetes

Sample Type: EDTA Whole Blood

Method: *High Pressure Liquid Chromatography*

Due to a high specificity and reproducibility, High Pressure Liquid Chromatography (HPLC) is considered to be the gold standard method for the determination of HbA1c. NOTE: Presence of Hemoglobin variants and/or conditions that affect red cell turnover must be considered particularly when the HbA1c results does not corelate with the patient's blood glucose levels.Hemoglobin variants and elevated fetal hemoglobin(HbF) can affect the accuracy of HbA1c measurements.Any condition that shortens red cell survival (hemolytic anemia, recovery from acute blood loss, pregnancy, HbSS, HbSC, HbSC) will falsely lower HbA1c results. Iron deficiency anemia is associated with higher HbA1c..

Complete Blood Count

Test Name	Result	Flag	Units	Ref. Val.
✓White Blood Cell	5.70		10^9/L	(4.0 - 10.0)
✓Neutrophils, Absolute	3.93		10^3/uL	(2.0 - 7.0)
✓Lymphocytes, Absolute	1.20		10^3/uL	(1.0 - 3.0)
✓Monocytes, Absolute	0.41		10^3/uL	(0.20 - 1.00)
✓Eosinophils, Absolute	0.08		10^3/uL	(0.02 - 0.50)
Basophils, Absolute	0.03		10^3/uL	Less than 0.10
✓% Neutrophils	69.55		%	(36.9 - 73.3)
✓% Lymphocytes	21.22		%	(20.0 - 40.0)
✓% Monocytes	7.29		%	(2.0 - 10.0)
✓% Eosinophils	1.44		%	(1.0 - 6.0)
✓% Basophils	0.50		%	Less than 2.0

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✓Red Blood Cells (RBC)	4.81		10^{12} /L	(4.50 - 5.50)
✓Hemoglobin(HBG)	14.50		g/dL	(13.0 - 17.0)
✓Hematocrit (HCT)	42.60		%	(40.0 - 50.0)
✓Mean Corpuscular Volume (MCV)	88.50		fL	(80 - 96)
✓Mean Corpuscular Hemoglobin (MCH)	30.20		pg	(27.0 - 33.0)
✓Mean Corpuscular Hb. Conc. (MCHC)	34.20		g/dl	(31.5 - 36.2)
✓RDW	12.70		%	(11.6 - 14.0)
✓Platelets	158.00		10^3/uL	(150 - 410)
Mean Platelet Volume	10.80		fL	(7.4 - 11.4)
Plateletcrit	0.17		%	(0.16 - 0.35)
Platelet Distribution Width	17.30			(16.0 - 17.6)
Nucleated RBC Count/100 WBCs	0.07			(0.0 - 0.1)
Nucleated RBC, Absolute	0.00		10^9/L	Less than 0.03
Early Granulated Cell %	0.17		%	(0 - 1)
Early Granulated Cell Absolute	0.01		10^9/L	(0.00 - 0.06)
Neutrophil-Lymphocyte Ratio (NLR)	**3.28**	H		(1.0 - 3.0)
Platelet-Lymphocyte Ratio (PLR)	131.67			(90 - 210)
Systemic Immune-Inflammation Index (SII)	517.45		10^9/L	(142 - 804)

Sample Type: EDTA Whlole Blood

Method: *Coulter principle: RBC,WBC, PLT. VCS 360 technology: DLC. Photometric measurement: HGB. All others derived/calculated*

Derived hematological indices and ratios reported in this section (including NLR, PLR, SII and other calculated indices) are intended as adjunctive parameters and should be interpreted in conjunction with the complete blood count, other relevant laboratory findings, clinical history, and clinical examination. These indices are not diagnostic when interpreted in isolation and should not be used as a sole basis for clinical diagnosis or management.

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Test Name	Result	Flag	Units	Ref. Val.
✓Sodium	139.00		mmol/L	(136 - 145)

Sample Type: Serum

Method: *Indirect Integrated Multisensor Technology (IMT)*

Due to changes in instrumentation / methodology as of 25/06/2024, please see updated reference range for Sodium.

✓Chloride	102.00		mmol/L	(98 - 107)

Sample Type: Serum

Method: *Indirect Integrated Multisensor Technology (IMT)*

Bromide, Salicylate, citrate, and Iodine may produce interferance with the test. Due to changes in instrumentation / methodology as of 25/06/2024, please see updated reference range for Chloride.

✓Urea	46.27		mg/dL	(19.3 - 49.2)
	7.73		mmol/L	(3.22 - 8.22)

Sample Type: Serum

Method: *Calculation*

Blood samples from some patients with monoclonal gammopathies may produce falsely elevated results. Due to changes in instrumentation as of 25/06/2024, please see updated reference range for Urea.

✓Creatinine	**1.20**	H	mg/dL	(0.60 - 1.10)
	106.20	H	umol/L	(53.10 - 97.35)

Sample Type: Serum

Method: *Enzymatic*

Interference has been observed with dobesilate and phenindione. Blood samples from some patients with monoclonal gammopathies (for example, Waldenstroms Macroglobulinemia) may produce falsely elevated results. Sample collection should be occur prior to N-Acetyl Cysteine (NAC) or Metamizole (Sulpyrine)administration due to the potential for falsely depressed results. Due to changes in instrumentation as of 25/06/2024, please see updated reference range for Creatinine.

✓BUN	21.62		mg/dL	(9 - 23)
	7.718		mmol/L	(3.213 - 8.211)

Sample Type: Serum

Method: *Roch-Ramel enzymatic reaction using urease*

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Test Name	Result	Flag	Units	Ref. Val.

Blood samples from some patients with monoclonal gammopathies may produce falsely elevated results. Due to changes in instrumentation as of 25/06/2024, please see updated reference range for BUN.

Serum BUN / Creatinine Ratio	18.02			(10 - 20)

Method: *Calculation*

An increased ratio may be due to a condition that causes a decrease in the flow of blood to the kidneys, such as congestive heart failure or dehydration. It may also be seen with increased protein, from gastrointestinal bleeding, or increased protein in the diet. The ratio may be decreased with liver disease (due to decrease in the formation of urea) and malnutrition. The reference range has been reviewed and updated as of 07-12-2024.

✓Uric Acid	7.20		mg/dL	(3.7 - 9.2)
	428.40		umol/L	(220.15 - 547.40)

Sample Type: Serum

Method: *Enzymatic reaction using uricase / peroxidase*

Collection of the sample should occur prior to N-Acetyl Cysteine (NAC) or Metamizole (Sulpyrine) administration due to the potential for falsely depressed results. Due to changes in instrumentation as of 25/06/2024, please see updated reference range for Uric Acid.

✓Total Protein	70.00		g/L	(57 - 82)

Sample Type: Serum

Method: *Biuret Colorimetric Method*

A potential interference in protein results for patients receiving dextran as blood volume expanders. This potential interference would appear as an overestimation or a false high in results. Due to changes in instrumentation / methodology as of 23/06/2024, please see updated reference range for Total Protein.

Albumin	44.00		g/L	(37 - 55)

Sample Type: Serum

Method: *Colorimetric Bromocresol Green*

Due to changes in instrumentation / methodology as of 23/06/2024, please see updated reference range for Albumin.

FIB-4 Index	0.85			

Sample Type: Serum

Method: *Calculation*

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Test Name	Result	Flag	Units	Ref. Val.

FIB-4 Score	Approximate Fibrosis Stage
<1.45	0-1
1.45-3.25	2-3
>3.25	4-6

The FIB-4 score is a non-invasive index used to estimate the degree of liver fibrosis in patients with risk factors for chronic liver disease, including chronic viral hepatitis, alcoholic liver disease, MASLD, and metabolic or cholestatic liver disorders. It may also be used to monitor fibrosis progression over time in patients with established liver disease.

A low FIB-4 score suggests a low likelihood of advanced fibrosis and may support conservative or medical management without the need for liver biopsy, particularly if values remain stable on serial testing. In contrast, a high FIB-4 score is suggestive of advanced fibrosis or cirrhosis and may warrant further evaluation, including consideration of liver biopsy, unless corroborated by clinical findings or imaging indicative of advanced liver disease.

Interpretation should be made in conjunction with clinical context, laboratory findings, and imaging studies. Caution is advised when applying this score in patients younger than 35 years or older than 65 years, as diagnostic accuracy may be reduced in these age groups.

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Test Name	Result	Flag	Units	Ref. Val.
Globulin	26.00		g/L	(16 - 36)
Sample Type: Serum				
Method: *Calculation*				
Albumin/Globulin (A/G) Ratio	1.69		Ratio	(0.7 - 2.80)
Sample Type: Serum				
Method: *Calculation*				
✓Alkaline Phosphatase	67.00		U/L	(46 - 116)
Sample Type: Serum				
Method: *IFCC Standardization Colorimetric p-Nitrophenyl Method*				
Due to changes in instrumentation / methodology as of 23/06/2024, please see updated reference range for Alkaline Phosphatase.				
✓Alanine Aminotransferase (ALT/SGPT)	29.00		U/L	Less than 49
Sample Type: Serum				
Method: *IFCC / Enzymatic without P5P*				
Sulfasalazine concentrations in serum or plasma above 50 mg/L may cause falsely depressed results. Due to changes in instrumentation / methodology as of 23/06/2024, please see updated reference range for Alanine Aminotransferase (ALT/SGPT).				
✓Aspartate Aminotransferase (AST/SGOT)	20.00		U/L	Less than 34
Sample Type: Serum				
Method: *IFCC / Enzymatic without P5P*				
The reference range has been reviewed and updated as of 03-09-2024.				
✓Bilirubin Total	**1.300**	H	mg/dL	(0.3 - 1.2)
	22.23	H	umol/L	(5.13 - 20.52)
Sample Type: Serum				
Method: *Vanadate oxidation*				
Due to changes in instrumentation / methodology as of 23/06/2024, please see updated reference range for Bilirubin Total.				
✓Bilirubin Direct (Conjugated)	**0.36**	H	mg/dL	Less than 0.3

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Test Name	Result	Flag	Units	Ref. Val.
	6.16	H	umol/L	Less than 5.13

Sample Type: Serum

Method: *Vanadate oxidation*

Due to changes in instrumentation / methodology as of 23/06/2024, please see updated reference range for Bilirubin Direct (Conjugated).

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Test Name	Result	Flag	Units	Ref. Val.
✓Gamma Glutamyl Transferase (GGT)	12.00		U/L	Less than 73

Sample Type: Serum

Method: *Modified IFCC Colorimetric Method*

Due to change of instrumentation as of 23/06/2024, please see updated reference range for Gamma Glutamyl Transferase (GGT).

✓Calcium	9.70		mg/dL	(8.3 - 10.6)

Sample Type: Serum

Method: *O-cresolphthalein complexone without deproteinization*

Due to changes in instrumentation / methodology as of 25/06/2024, please see updated reference range for Calcium.

C- Reactive Protein	0.60		mg/L	Less than 8.0

Sample Type: Serum

Method: *Latex-enhanced turbidimetric*

Measuring changes in the concentration of CRP provides useful diagnostic information about how acute and how serious a disease is. Persistance of a high serum CRP concentration is usually a prognostic sign which generally indicates the presence of an uncontrolled infection.

✓C - Reactive Protein (CRP- High Sensitivity)	0.68		mg/L	Less than 3 (Healthy individuals) Less than 1 (Low risk for cardiovascular disease prediction) (1 - 3) Average risk for cardiovascular disease prediction More than 3 (High risk for cardiovascular disease prediction)

Sample Type: Serum

Method: *Turbidimetry method*

Due to changes in instrumentation / methodology as of 16/05/2024, please see updated reference range for C - Reactive Protein (CRP- High Sensitivity).

Estimated Average Glucose	108.28		mg/dl

Method: *Calculation*

The normal value for estimated average glucose is between 70 mg/dl and 126 mg/dl (A1C: 4% to 6%).Due to changes in instrumentation / methodology, as of 29/09/2021,please see updated unit and reference range for estimated average glucose.

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Fasting: Yes		

Test Name	Result	Flag	Units	Ref. Val.
✓Glucose - Fasting (FBS)	64.00		mg/dL	(70 - 100) Normal (100 - 125) Prediabetes More than 125 Diabetes
	3.52		mmol/L	(3.85 - 5.50) Normal (5.50 - 6.88) Prediabetes More than 6.88 Diabetes

Sample Type: NaF Blood

Method: *Hexokinase*

Triglyceride - Glucose Index	4.31			Less than 4.68

Sample Type: Serum and Sodium Fluoride

Method:

The Triglyceride-Glucose Index is a surrogate marker of insulin resistance. Interpretation should consider clinical context and other metabolic risk factor. The reference range has been reviewed and updated as of 19-08-2025.

Test Name	Result	Flag	Units	Ref. Val.
✓Iron	**234.00**	H	µg/dL	(65 - 175)

Sample Type: Serum

Method: *Ferrozine Colorimetric Method*

✓Total Iron Binding Capacity (TIBC)	280.00		µg/dL	(250 - 425)

Sample Type: Serum

Method: *Chromazuro Colorimetric*

Due to changes in instrumentation / methodology as of 22/06/2024, please see updated reference range for Total Iron Binding Capacity (TIBC).

✓Transferrin (Iron) Saturation	**83.57**	H	%	(20 - 50)

Sample Type: Serum

Method: *Calculation*

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Test Name	Result	Flag	Units	Ref. Val.
✓Cholesterol	**210.00**	H	mg/dL	Less than 200 Desirable (200 - 239) Borderline High More than 240 High
	5.46	H	mmol/L	Less than 5.20 Desirable (5.20 - 6.21) Borderline High More than 6.24 High

Sample Type: Serum

Method: *Enzymatic Colorimetric Method*

The reference range has been reviewed and updated as of 20-02-2023.

✓HDL - Cholesterol	48.80		mg/dL	More than 40 Desirable
	1.27		mmol/L	More than 1.04 Desirable

Sample Type: Serum

Method: *Homogeneous enzymatic colorimetric test*

According to ATP-III Guidelines, HDL-C >59 mg/dL is considered a negative risk for CHD.

Non HDL - Cholesterol	**161.20**	H	mg/dL	Less than 130 Desirable More than 130 High Risk
	4.19	H	mmol/L	Less than 3.38 Desirable More than 3.38 High Risk

Method: *Calculation*

According to ATP-III Guidelines, HDL-C >59 mg/dL is considered a negative risk for CHD.

✓Triglycerides	86.00		mg/dL	Less than 150 Acceptable (150 - 199) Borderline high (200 - 499) High More than 500 Very high
	0.946		mmol/L	Less than 1.650 Acceptable (1.650 - 2.189) Borderline high (2.200 - 5.489) High More than 5.500 Very high

Sample Type: Serum

Method: *Enzymatic Colorimetric Method*

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Test Name	Result	Flag	Units	Ref. Val.
✓LDL - Cholesterol, Calculated	**146.10**	H	mg/dL	Less than 100 Desirable (100 - 129) Above Desirable (130 - 159) Borderline High (160 - 189) High More than 190 Very High
	3.80	H	mmol/L	Less than 2.60 Desirable (2.60 - 3.35) Above Desirable (3.38 - 4.13) Borderline High (4.16 - 4.91) High More than 4.94 Very High
Method: *Calculation (Modified Sampson Equation)*				
Updated methodology as of 31-07-2026.				
✓Very Low Density Lipid (VLDL)	17.20		mg/dL	(0 - 40)
	0.45		mmol/L	(0.00 - 1.04)
Method: *Calculation*				
Cholesterol/HDL Cholesterol Ratio	**4.30**	H		Less than 3.5 Normal
Method: *Calculation*				
LDL Cholesterol/HDL Cholesterol Ratio	2.99			Less than 3 Normal
Method: *Calculation*				
Triglycerides/HDL Cholesterol Ratio	1.76			Less than 2.0 Normal
Method: *Calculation*				

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Name: ALEXANDER WILLIAM PETER THORP JIGGINS	Collection Date : 13/Oct/2026 09:10:00	Doctor: REFERRAL (VALEO WELLBEING DUBAI)
Sample Id: D26286061	Passport No:	Emirates ID: 784199020639185
Patient Id: MR01330101	Nationality: BRITISH	Center: VALEO WELLBEING DUBAI
Age/Sex: 36 Years/Male	Telephone: 971000000000	Clinic Barcode : VALEO-219231
DOB: 10/Jan/1990	File No: 2303	E-mail:
Fasting: Yes		

Test Name	Result	Flag	Units	Ref. Val.
Estimated Glomerular Filtration Rate	80		ml/min/1.73m2	Less than 15 CKD Stage V : Kidney failure (15 - 30) CKD Stage IV : Severe decrease in GFR (30 - 60) CKD Stage III : Moderate decrease in GFR (60 - 90) CKD Stage II : Mild decrease in GFR More than 90 CKD Stage I : Normal or Minimal Kidney Damage with normal/high GFR

eGFR between 60-90 in absence of evidence of kidney damage should be repeated in 3 months to check that it remains lower before considering CKD. Please note that some factors can affect eGFR including: pregnancy, being over the age of 70, unusual muscle mass, cirrhosis, nephrotic syndrome, a past solid organ transplant, and some medications.

Method: *Calculation*

Estimated GFR calculated using the 2021 CKD-EPI creatinine equation is not valid for patients younger than 18 years old.
Results must be interpreted along with quantitative urine albumin-creatinine ratio in parallel with the eGFR.

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Test Name	Result	Flag	Units	Ref. Val.
Ferritin	84.50		ng/mL	(10.5 - 307.3)

Sample Type: Serum

Method: *CLIA*

Serum ferritin may be elevated due to non-iron related causes including systemic infection or inflammation, tissue injury, liver disease, malignancy (e.g., acute leukemia, Hodgkin's disease),and recent or ongoing iron supplementation. In such settings, ferritin levels may not accurately represent true body iron stores. Presence of Heterophile antibody in the sample may interfere with test result. Correlation withiron indices and clinical context is recommended. The American Society of Hematology (ASH) 2026 Clinical Practice Guidelines recommend a serum ferritin threshold of less than or equal to 30 ng/mL for the diagnosis of iron deficiency in adults, menstruating individuals, and pregnant individuals.A threshold of less than or equal to 50 ng/mL may be appropriate in individuals at increased risk of iron deficiency, including those with heavy menstrual bleeding or pregnancy-associated anemia. In adults with inflammation, iron deficiency may be considered when serum ferritin is less than 100 ng/mL or transferrin saturation (TSAT) is less than 20%.These thresholds are clinical decision limits and should be interpreted in the appropriate clinical context; ferritin results should not be interpreted in isolation, particularly in the presence of inflammation. The reference range has been reviewed and updated as of 31-01-2026.

Vitamin B12	207.00	pmol/L	(156 - 672)
	280	pg/mL	(211 - 911)

Sample Type: Serum

Method: *CLIA*

1st Trimester Pregnancy: 87-323 pmol/L (118-438 pg/mL)

2nd Trimester Pregnancy: 93-484 pmol/L (130-656 pg/mL)

3rd Trimester Pregnancy: 73-388 pmol/L (99-526 pg/mL)

Recent vitamin B12 administration could result in normal or elevated serum concentrations; therefore, this test should not be ordered on patients who have received a vitamin B12 injection within the past 2 weeks. Other factors could increase B12 level include : ingestion of Vitamin C, Estrogen and Vitamin A, Myeloproliferative disorder, Hepatocellular injury and Uremia.A recent metanalysis of more than 300 thousand patient samples has concluded that fasting is not required for evaluating Vitamin B12 levels (Fasting time and vitamin B12 levels in a community-based population doi: 10.1016/j.cca.2016.05.005. Epub 2016 May 10. PMID: 27178484).Excessive exposure of samples to light may alter vitamin B12 values. Due to changes in instrumentation / methodology as of 06/07/2024, please see updated reference range for Vitamin B12.

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Test Name	Result	Flag	Units	Ref. Val.
Vitamin D Total (25-Hydroxycholecalciferol)	36.01		ng/mL	Less than 20 Deficiency (20 - 30) Insufficiency More than 30 Adequate
	90.0		nmol/L	Less than 50.0 Deficiency (50.0 - 75.0) Insufficiency More than 75.0 Adequate

Sample Type: Serum

Method: *CLIA*

Do not use samples that contain fluorescein. Fluorescein levels > 0.10 µg/mL can produce falsely elevated results in this assay. The reference range has been reviewed and updated as of 08-10-2024.

Test Name	Result	Flag	Units	Ref. Val.
✓Estradiol (E2)	30.20		pg/mL	Less than 39.8
	110.83		pmol/L	Less than 146.07

Sample Type: Serum

Method: *CLIA*

The drug Fulvestrant (FASLODEX®) may cause falsely elevated estradiol results in immunoassays. For patients being treated with Fulvestrant. With the advent of new steroid-based medications (analogues) with similar chemical. Samples should not be taken from patients receiving therapy with high biotin or vitamin B7 doses until at least 12 hours following the last biotin administrationfor due to falsely increased results. Due to changes in instrumentation / methodology as of 28/06/2024, please see updated reference range for Estradiol (E2).

Follicle Stimulating Hormone (FSH)	5.01	IU/L	(1.4 - 18.1)

Sample Type: Serum

Method: *CLIA*

Due to changes in instrumentation / methodology as of 28/06/2024, please see updated reference range for Follicle Stimulating Hormone (FSH).

Luteinizing Hormone (LH)	2.91	IU/L	(1.5 - 9.3)

Sample Type: Serum

Method: *CLIA*

Due to changes in instrumentation / methodology as of 28/06/2024, please see updated reference range for Luteinizing Hormone (LH).

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Fasting: Yes		

Test Name	Result	Flag	Units	Ref. Val.
Insulin - Fasting	8.15		uIU/mL	Less than 25.00

Sample Type: Serum

Method: *CLIA*

Insulin autoantibodies in human serum may interfere and cause discordant results.

HOMA IR Index	1.29		uIU/mL	Less than 2.0 Insulin resistance unlikely (2.0 - 2.5) Possible insulin resistance More than 2.5 Insulin resistance likely

Method: *Calculation*

Ref: http://jmscr.igmpublication.org/v4-i9/65%20jmscr.pdf

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Fasting: Yes		

Test Name	Result	Flag	Units	Ref. Val.
✓Thyroid Stimulating Hormone (TSH)	2.87		uIU/mL	(0.55 - 4.78)

Sample Type: Serum

Method: *CLIA*

Falsely elevated TSH results can be caused by: Heterophile antibodies or non-specific proteins in patients exposed to animals or animal serum products, that can react adversely with reagent. Fluorescein levels >0.24 µg/mL may decrease results in this assay. Evidence suggests that patients undergoing retinal fluorescein angiography can retain amounts of fluorescein in the body for up to 48-72 hours post-treatment. It is recommended to repeat TSH test after suitable interval to rule out transient/false elevations. Due to changes in instrumentation / methodology as of 29/06/2024, please see updated reference range for Thyroid Stimulating Hormone (TSH).

Tri-Iodothyronine-Free (FT3)	0.34	ng/dL	(0.23 - 0.42)
	5.22	pmol/L	(3.53 - 6.45)

Sample Type: Serum

Method: *CLIA*

Due to changes in instrumentation / methodology as of 28/06/2024, please see updated reference range for Tri-Iodothyronine-Free (FT3).

Thyroxine - Free (FT4)	1.17	ng/dL	(0.89 - 1.76)
	15.06	pmol/L	(11.46 - 22.65)

Sample Type: Serum

Method: *CLIA*

The anticonvulsant drug phenytoin may interfere with total and free T4 levels due to competition for TBG binding sites. Free T4 values may be decreased in patients with non-thyroidal conditions and in patients taking carbamazepine. Thyroid autoantibodies in human serum may interfere and cause falsely elevated free T4 results. Biotin more than 3500 ng/mL may interfere with the test. Due to changes in instrumentation / methodology as of 29/06/2024, please see updated reference range for Thyroxine - Free (FT4).

Testosterone Total	5.31	ng/mL	(2.64 - 9.16)
	18.43	nmol/L	(9.16 - 31.79)

Sample Type: Serum

Method: *CLIA*

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Fasting: Yes		

Test Name	Result	Flag	Units	Ref. Val.

Please note that AM levels are higher than PM levels. The androgens status should includemore than one single measurement as testosterone levels fluctuate. A strong interaction with Nandrolone decanoate, 11B-hydroxytestosterone, and 11-ketotestosterone was found. Do not use samples from patients receiving these compounds. With the advent of new steroid-based medications (analogues) with similar chemical structures to testosterone, there is the possibility of cross-reactivity and falsely elevated results.Samples with biotin concentrations > 30 ng/mL may cause erroneous results. Rheumatoid factor more than 200 IU/mL can be affect on the test. More than 30 ng/mL of biotin can affect on the test. The reference range has been reviewed and updated as of 22-08-2024.

Test Name	Result	Flag	Units	Ref. Val.
CA 15-3 (Breast)	12.20		U/mL	Less than 32.4

Sample Type: Serum

Method: *CLIA*

Values obtained with different assay methods cannot be used interchangeably. Do not interpret serum levels of CA 15-3 as absolute evidence of the presence or the absence of malignant disease. Elevated levels of CA 15-3 can be observed in patients with non-malignant diseases. Measurements of CA 15-3 should always be used in conjunction with other diagnostic procedures, including information from the patient's clinical evaluation. Exercise caution when interpreting CA 15-3 levels during pregnancy. Test performed on Siemens Atellica SCI/ CI-1900. Due to changes in instrumentation / methodology as of 07/07/2024, please see updated reference range for CA 15-3 (Breast).

Urine Analysis

Physical Examination

Test Name	Result	Flag	Units	Ref. Val.
✓Appearance	Clear			Clear
✓Color	Yellow			Pale Yellow

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Chemical Examination

✓Specific Gravity	1.021	(1.005 - 1.030)
✓pH	6.5	(4.6 - 8.0)
✓Protein	Trace	Negative
✓Glucose	Negative	Negative
✓Ketone	Negative	Negative
✓Urobilinogen	Normal	Normal
✓Bilirubin	Negative	Negative
✓Blood	Negative	Negative
✓Leukocyte	Negative	Negative
✓Nitrite	Negative	Negative

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Microscopic Examination

✓Pus Cells	0-2	/HPF	0-5
✓Red Cells	0-2	/HPF	0-3
✓Epithelial Cells	0-1	/HPF	0-10
✓Casts	Negative		Negative
✓Crystals	Negative		Negative
✓Bacteria	Negative		Negative
✓Yeast	Negative		Negative
✓Others	Negative		Negative

Sample Type: Urine

Method:*Colour/Appearance-Light scatter, Specific Gravity-Refractometry, pH- Bromothymol blue, Glucose: GOD/POD Chromogen, Ketone-Legal's reaction, Protein- Tetra-bromophenol blue, Blood-Peroxidase activity, Bilirubin- Acidified Diazo coupling, Urobilinogen-Buffered Diazo coupling, Nitrate-Modified Gries Reaction, Leukocytes-Esterase/Enzymatic.Microscopy by: Digital Cell Morphology/Auto Particle Recognition*

Remarks: Presence of numerous epithelial cells (more than 10), bacteria with positive nitrate and negligible pus cells (less than 5) are most likely indicative of contamination. Please correlate clinically. Presence of bacteria does not indicate infection and it can be a contamination and should be correlated clinically. Due to change in methodology, the reference range has been reviewed and updated as of 17-11-2023.

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Stool Analysis

Macroscopic Examination

Test Name	Result	Flag	Units	Ref. Val.
✓Consistency	Formed			Well Formed
✓Color	Brown			Light to Dark Brown
✓Blood	Nil			Nil
✓Mucous	Nil			Nil
✓Parasites	Nil			Nil

Microscopic Examination

Method: Wet mount

✓Pus Cells	0-2			0-1
✓RBC`s	Nil			Nil
✓Ova	Nil			Nil
✓Cysts	Nil			Nil
✓Trophozoites	Nil			Nil
✓Yeast Cells	Nil			Nil
✓Undigested Food	Nil			Nil
✓Fat Globules	Nil			Nil
✓Others	Nil			Nil

Sample Type: Stool

The stool wet mount examination is a preliminary microscopic screening test for detecting motile parasites, cysts, and ova. Results depend on specimen quality, timing, and parasite concentration. A negative result does not exclude parasitic infection. For higher accuracy, it is recommended to submit at least three stool samples on separate days or perform confirmatory testing (e.g., concentration, staining, or antigen/molecular methods) when clinically indicated.

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